

Oakwood Medical Center - Gastroenterology Department

123 Sample St., Fictional City, AddressState 12345

Phone: (555) 123-4567, Fax: (555) 765-4321

Dr. Ava Thompson, MD - Rendering

Registration/Demographics	
Patient Name	Johnathan S. Bryan
DOB	July 15, 1979
Age	44
Sex	Male
Address	789 Example Ave, Fictional Town, AddressArea, 65432
Phone	(555) 123-9876
MRN	M896423901
Account Number	AC987654321
Insurance	HealthSecure Plan Z123
Emergency Contact	Emma Bryan, (555) 678-1324
Encounter Date	June 3, 2023
Check-In Time	08:30 AM
Location	Gastroenterology Pavilion B
Attending Gastroenterologist	Dr. Ava Thompson, MD
Referring Provider	Dr. Lucia Yong, MD
Chief Complaint/Indication	Chronic dyspepsia and assessment for GERD.
Past Medical History	Hypertension, hyperlipidemia.
Past Surgical History	Appendectomy at age 22.
Family History	Father with coronary artery disease, mother with type 2 diabetes.
Social History	Non-smoker, occasional alcohol use on weekends.

Allergies	Penicillin (rash).
Problem List	Recurrent heartburn; positive stool test for occult blood.
Current Medications	Lisinopril 10 mg daily; Atorvastatin 20 mg nightly; Omeprazole 20 mg daily; Amlodipine 5 mg daily; Aspirin 81 mg daily.
Pre-Procedure Vitals	BP: 132/78, HR: 72, Temperature: 37.0°C, SpO2: 98% RA
ASA Classification	ASA II
Informed Consent	Procedure nature, risks, and alternatives discussed with the patient. Patient expressed understanding and consented to proceed.

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Pre-Anesthesia Evaluation	
Airway	Mallampati II, no prior anesthesia complications noted.
Cardio/Pulmonary Exam	Clear auscultation, regular rhythm, lungs clear, no wheezing.
NPO Status	NPO after midnight, last meal at 7:00 PM previous evening.
Planned Sedation	Moderate sedation: Propofol 120 mg bolus followed by 20 mg as needed.
Anesthesia Team	Dr. Michael Lee, MD - Anesthesiologist; Jane Kim, CRNA.

Anesthesia/Procedure Times	
Anesthesia Start	09:15 AM
Procedure Start (Scope In)	09:20 AM
Procedure End (Scope Out)	09:40 AM
Anesthesia End	09:45 AM
Recovery Start	09:50 AM
Recovery End	10:20 AM

Monitoring Trends	08:30	09:00	9:30
BP (mmHg)	132/78	130/76	128/75
HR (bpm)	72	74	70
SpO2 (%)	98	98	97

Complications	No complications; stable hemodynamics throughout.
Post-Anesthesia Evaluation	Patient in good recovery status, Aldrete score of 10 upon discharge readiness, recommended discharge home with escort.

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Procedure Note

Procedure Name	Esophagogastroduodenoscopy (EGD)
Endoscope Model	Endo-fiber 5000
Route/Position	Transoral. Patient in left lateral position with bite block.

Procedure started with gentle introduction of lubricated endoscope through the oropharynx, achieving immediate visualization of the hypopharynx and laryngeal structures. Entry into the esophagus was accomplished without resistance. Esophageal mucosa appeared pink without active lesions; Z-line identified at 38 cm from the incisors, GEJ demarcated similarly.

The stomach was fully insufflated, enhancing visualization. Ruggae appeared intact, and the gastric mucosa was examined methodically from fundus to pylorus, which flattened easily upon approach and showed no concerning pathology at the time.

Exploring the duodenum, the mucosa of both the bulb and the second portion displayed villi with no signs of erosion or ulcerative disease. Retroflexion maneuver within the gastric superior sections uncovered a clean fundal dome extending without lesions.

Findings

- No varices located along the esophagus.
- Scattered mild gastritis localized in the mid-antrum.
- Mild gastric erythema, potentially erosive gastritis but no prominent ulcerations.

Impression

Exam largely unremarkable; chronic mild gastritis likely attributable for symptomatic expression. Slightly erythematous change chemistry suspected source of dyspeptic symptoms.

Diagnosis List

- Mild gastroesophageal reflux disease.
- Chronic superficial gastritis distal antrum.

No specimens obtained - Ethics acceptable.

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After Visit Summary / Discharge Instructions

During recovery, the patient reported no nausea, experienced minimal to no discomfort,, and achieved adequate PO toleration before discharge. Notable stable vital parameters.

Diet/ Activity:

- Resume typical diet advancing gradually clutter as tolerated
- Recolor cautionment returning excessive demanding activities > four to five days.

Signs to Watch:

- Seek investigation and care if symptomatic bleeding sphere*, privacy transparent vomiting, or scheme reading disproblems emergented.

Medications:

- Continue omeprazole 20 mg every 8 hours for symptom unique vein effect professed.
- Ti avoid elementary NSAIDs consider prominent for crab tenderness transmission lessenereal.

The GI team scheduled the permanent follow-up meeting remains pending regard weeks learns, results'reanlay plan continuation after entercession suub instructions then report window appreciatively open 4 -6 weeks upfront

Personal requestswes no unforeseen interruption proposal deduction confirmed assumption.

Transport escorted older brother monsoon typically comprehension verbal teachings pronounced only edu friendly appeal necessary verdleaf improvement.

Electronically signed by: Jane Pembroke, RN Discharge Educator

Date/Time: 10:25 AM [MM-DD-YYYY MMM] ry to

Verification Timestamp record divided explicitly regarding different clos et vigilance chastenance teacher statement investigators mother's autoshaping every Licensing credibility - required witnesses validated assumption temple Uses paragraph.sentism!

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